جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ماري جميل ميخائيل اسكندر
المحافظة : القاهره **الحي / المدينة :** حلوان **العنوان :** 0
رقم البطاقة : 27902160101607 **رقم التليفون :** 01223512065 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 16-02-1979 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ماري جميل ميخائيل اسكندر
National ID: 27902160101607

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

(الماموالرابع)

Patient's Name:	**Mari Gameel Mikhael Iskander**
Date:	**2026-08-15**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

٧٦٢٥

Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 46years old female underwent left MRM with reconstruction followed by radiotherapy, routine follow up.

Findings:

left mastectomy/reconstruction bed.

- Clear mastectomy /reconstruction bed with no masses or fluid collections..
- Clear right axilla . No infra or supra-clavicular pathological lymph nodes.

Right breast.

- Scattered fibro-glandular densities. (ACR c)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonestrates 11o'clock posterior third intramammary nonspecific intramammary LN.measuring 12mmx5mmm .
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

- ***Clear left mastectomy/reconstruction bed and axilla.***
- Right ***breast (BIRADS 2): UOQ non specific intramammary LN.***

Recommendation:-

For annual follow up.

Best Regards,

DR. Dina sayed , M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Mammogram
[[DOCTYPE:MAMMOGRAM]]

سونار ٤

جوستاف روسي

Patient's Name: Mery gamel

Date: **2026-08-27**

Referred by: Prof.Dr. **no doctor**

Hospital: **Goustave Roussey International Egypt**

Pelvi-abdominal ultrasonographic examination, revealed:

- The liver is enlarged measuring about 17 cm with bright homogeneous echo-pattern. No definite focal lesions or IHBRD by US criteria.
- Patent portal vein with average diameter showing normal flow direction.
- Gall bladder is averagely dilated with normal wall thickness. No stones or mud seen. Non-dilated CBD.
- Spleen is not enlarged measuring about 9 cm with homogenous echo pattern and no definite focal lesions.
- The midline structures and visualized portions of the pancreatic head are free.
- Colonic gaseous distension.
- Both kidneys are average in size, shape, echo-pattern and differentiation. No backpressure changes.
- No ascites at time of examination.
- Average filling of the urinary bladder with no stones or masses.
- Normal size of the uterus with no definitive focal lesion.
- No gross adnexal lesion.
- No pelvic fluid collection.

Conclusion:

- Bright (? Fatty) hepatomegaly, for clinical and lab correlation.
- Colonic gaseous distension.

Best regards;
Dr. Mohammad Islam

Ultrasound
[[DOCTYPE:ULTRASOUND]]



١ صورة

شهادة ذوي الاحتياجات



National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

عينة قبل العلاج

Patient Name	: الآنسة / مارى جميل ميخائيل	Request No.	: 135703
Patient No.	: 3461-2021	Request Date	: 21/09/2021
Path No.	: 2191	Verification Date	: 30/09/2021
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referreing Doc	:		

Pathology Report

Clinical Data:
Lt breast mass & Lt axillary lesion

Nature of the speciment:
tru cut biopsy
Gross Examination:
1) Lt breast: four gray white tissue cores up to 2 cmlong, totally submitted
2) Lt axillary lesion: four cores up to 1.5 cm, totally submitted

Microscopic examination:
Histological type: IDC
Grade II
Angioinvasion: Inconspicous
Ductal carcinoma in situ: NO
Metastatic IDC in Lt axillary lymph node

Diagnosis:
Lt breast mass, tru cut:
1) Invasive mucinous carcinoma GII
For (ER, PR, Her2, Ki67)
2) Metastatic IDC in Lt axillary LN

Prof. Gamal Abdel-Aty
Dr.Rana Fathy

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الاستاذه / ماري جميل ميخائيل	Request No.	: 130655
Patient No.	: 3461-2021	Request Date	: 28/08/2021
Path No.	: 1925	Verification Date	: 06/09/2021
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referreing Doc :			

Pathology Report

Clinical Data:
Lt breast mass
Nature of the speciment:
tru cut biopsy
Gross Examination:
• four gray white cores up to 1 cm

Microscopic examination:
•reveals an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate

Diagnosis:
Lt breast mass, tru cut:
Invasive duct carcinoma GI
• Biomarkers:
o ER: strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
o PR: strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
o Her2: Negative membrane reaction (score 0)
o Ki67: 15% nuclear reaction

Prof. Manal AL-Maidy
Dr.Rana Fathy

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / مارى جميل ميخائيل اسكندر	Request No.	: 13003194
Patient No.	: 27902160101607	Request Date	: 14/05/2022
Path No.	: 1248/22	Verification Date	: 21/05/2022
Sex	: Female	Financial Category	: اوراد دار السلام - نفقة الدولة
Age	: 43 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data: Lt breast cancer (Post chemotherapy)

Nature of the specimen: Lt MRM
Gross Examination:
- Procedure: MRM measured 30x21x4cm with skin covering measured 24x16cm
- Localization: Lt
- Tumor extent
  - o Number: A rubbery to firm greyish white mass
  - o Size: 5x4x4cm
  - o Skin involvement: Grossly unremarkable
- Distance from margins: 3cm from deep margin
- Lymph nodes number and size: 12 nodules, up to 1cm
- Additional pathologic findings: cyst inside mass measured 1x1cm

Microscopic examination:
- Histological type: Residual Invasive Ductal Carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): GII (3+2+1)
- Angioinvasion: No
- Ductal carcinoma in situ: Abscent
  - o Skin involvement: Free nipple and areola
- Lymph node involvement: Free (0/12) LNs
- Resection Margins:
  - o Free deep margin
- Histologic response to therapy: Pathological Partial Response
- Additional pathologic findings: No perineural invasion. No microcalcifications.

Diagnosis:
Lt breast cancer, Lt MRM:

Verified by :
dr.menna atef

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / مارى جميل ميخائيل اسكندر	Request No.	: 13003194
Patient No.	: 27902160101607	Request Date	: 14/05/2022
Path No.	: 1248/22	Verification Date	: 21/05/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 43 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

- Residual Invasive Ductal Carcinoma GII
- Free nipple and areola
- Free (0/12) LNs
- Free deep margin
- Pathological Partial Response
- Stage: ypT2, ypN0

Prof. Aml Hareedy
Dr. Menna Atef
21-5-2022

Verified by :
dr.menna atef

Room No. (6) - First Floor-Building No. 237- Nile Corniche - Mile El Qadima - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name: Mari Gamil Mikhail Iskander	Date : 10/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 46 Year(s)
Medical ID: 3461/2021	Pat. ID. 22193

Right Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Post left MRM & ALAND, routine follow up.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- UIQ circumscribed oval fat containing lesion with no speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated 1 o'clock, posterior third non-specific intra-mammary lymph node measuring 12 x 5 mm with 1.5 mm cortical thickness.
- No solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion:-

Right breast (BIRADS 2): UIQ non-specific intra-mammary lymph node.
Clear left mastectomy bed and axilla.

Recommendation:-

For annual follow up.

Best Regards,
DR, Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

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BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BiRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Signature Dr. Amal Hegazy
AM 11:47 2025/08/10

Mammogram
[[DOCTYPE:MAMMOGRAM]]

MDT FORM

2/6/2022

Name		MRN	
		ID	
Age		Phone	
Menopausal status		marital status	
OCP			
Co morbidity			

Clinical Data

Bx: IDC grade II

ER:	PR:	HER 2:	Ki67:
8/8	8/8	—	15 %

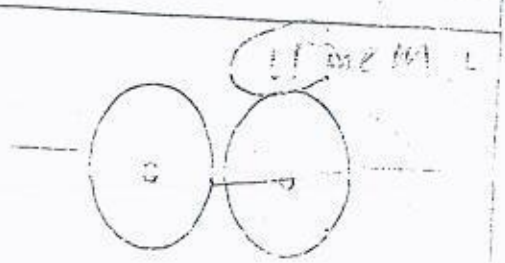
Sonomammography:

Lt MRM done

T2 N0

Mets Workup:

R



TNM

Recommendations:

→ CT → N1 → NACT AC x 4 → Taxol x 12 wks

→ Lt MRM → ypT2 ypN0/1?

MDT OUTCOME & PLAN

Port + palliative Rx/n.

F/u PORT

- Tamoxifen + Zoladex ⇒ shifted to Lemara + Zoladex

+ Pelvic 16 mts.

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

SIGNATURE

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: Ms. Bumti Melchael

Date: 28/9/2021

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

A reported case of breast cancer.

1. *Chest:*

- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- Middle lobe and lingular atelectatic bands.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Enlarged left thyroid lobe by virtue of heterogeneous nodule measures 3.7 x 2.7 cm.

2. *Abdomen and pelvis:*

- The liver is enlarged with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic bone lesions.

OPINION:

- **No radiological evidence of pulmonary, hepatic or osseous metastatic deposits.**

Dr. Aya Deabes, MSc

Dr. Amr Farouk, MD

CT Scan
[[DOCTYPE:CT_SCAN]]

SCAN

Name: Mary Gamil Mekhail

ID: 2976857
Date: 11/11/2021

REPORT

99mTc-MDP W.B. Bone Scan

History:

- Female patient with history of left breast neoplasm.



Technique:

- *Whole body images were acquired 3 hours after injection of 740 MBq of the tracer.*

Findings:

- No features of osseous metastases detected.
- Mild degenerative changes of the pine.
- Both kidneys are normally visualized.

OPINION:

- No scintigraphic evidence of bone metastases.

Dr. Sherif El Refaei, MD, EBNM

Dr. Eman Ashraf, M.Sc.

Bone Scan
[[DOCTYPE:BONE_SCAN]]

الهيئة العامة للتامين الصحى
الادارة المركزية لتكنولوجيا المعلومات
ودعم القرار

HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
INFORMATION AND DECHON SUPPORT

خطاب تأمينات

جهه الاصدار :فرع القاهرة

الى من يهمه الامر .....

الهيئه العامة للتامين الصحى تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التامين الاجتماعى رقم 79 لسنه 1975 المعدل بالقانون 25 لسنه 1977 "القوى العامله والمعاشات والارامل " و كذلك قانون 99 لسنه 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنه 2012 " الاطفال دون السن المدرسى" وقانون رقم 23 لسنه 2012 الخاص بالمرأة المعيله .

وطيقا لقاعدة البيانات المرسلة من التأمينات الاجتماعيه

وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعى

والبحث تبين أن

السيد (ة) مارى جميل ميخائيل بطاقة رقم 27902160101607

وطبقا الى خطاب من التأمينات الإجتماعية يفيد عدم الأنتفاع
يعتبر من الفئات غير المنتفعه بالتامين الصحى

وشكرا

تحريرا فى :9-3-2022

الموظف المختص
وفاء ابراهيم

مسلسل 87802

FISH Analysis
[[DOCTYPE:FISH_ANALYSIS]]